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ACADEMIC VAULT

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MED 101: Introduction to Medicine - History Taking, Ethics and Professionalism

The human and professional foundations of medical training in Nigeria

FACULTY PRIMER

Faculty Primer

Faculty of Medicine
Community Medicine & Primary Health Care | 100L

Written for students, by students.

Practical notes, worked examples, practice questions and a revision plan.

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Independent study support | Not an official UNIZIK publication

START HERE

Why this pack exists

Medical school begins with learning how to listen. This pack introduces patient history taking, communication, confidentiality, consent and the Nigerian medical code of conduct, along with a practical study routine for a course load that punishes passive reading.

This is a student-written study companion, not a reproduced lecture handout or a leaked examination paper. Use it to organise the topics you are taught, practise the kind of thinking your course rewards, and walk into class with better questions.

What you will be able to do

- Take a structured patient history using a clear sequence.
- Apply the four principles of biomedical ethics to common clinical scenarios.
- Explain confidentiality, informed consent and professional boundaries.
- Build a sustainable study routine for high-volume medical content.

How to use this pack

1. Skim the whole pack once in 20 minutes so you know what is inside.
2. Work through one section at a time with your lecture note and recommended text beside you.
3. Attempt the practice questions before reading the answers. Mark what you missed.
4. Use the final checklist to plan your revision week, one tick per study session.

> **Free to download, always**
> This pack is free. There is no fee, no subscription and no payment step before download, and the full PDF is not watermarked or locked. Share the link with a coursemate instead of forwarding the file.

> **Confirm your official course outline**
> Course codes, credit units and outlines can differ by department and session. Treat this pack as supplementary support and confirm the current outline with your lecturer, course rep or departmental handbook before an assessment.

Structure of a history

A complete history is a method, not a conversation you improvise. Practice the sequence until it becomes habitual.

1. Bio-data and presenting complaint, in the patient's own words plus duration.
2. History of presenting complaint using the mnemonic for the relevant symptoms.
3. Past medical and surgical history, drug history, allergies.
4. Family, social and occupational history.
5. Review of systems.

6. Summarise back to the patient and confirm that you understood correctly.

> **Open then closed**
> Begin with open questions such as 'tell me what brought you in', then narrow with closed questions. Jumping straight to closed questions produces incomplete histories.

Ethics that will actually be tested

Ethical reasoning is a subject with principles, not a personality trait.

- Autonomy: respect the patient's right to decide, supported by adequate information.
- Beneficence: act in the patient's best interest.
- Non-maleficence: avoid avoidable harm.
- Justice: treat similar cases similarly and use scarce resources fairly.
- Confidentiality: information is shared only with those involved in care or as legally required.
- Informed consent requires capacity, adequate disclosure, understanding and voluntariness.

Professional behaviour and self-management

Studies show that unprofessional behaviour in training predicts later problems. Start the habits now.

- Arrive early, dressed appropriately, with the tools you need.
- Introduce yourself to patients and ask permission before examination.
- Do not discuss patients in corridors, lifts or on WhatsApp.
- Document honestly and promptly. A wrong note left uncorrected can harm a patient.
- Study with spaced repetition and active recall rather than rereading slides.
- Protect sleep, exercise and relationships - burnout damages clinical judgement.

> **The code of conduct**
> Read the current Nigerian Medical and Dental Council code rather than relying on hearsay. Know the sections on confidentiality, advertising and relationships with colleagues.

PRACTICE, NOT PREDICTION

Check your understanding

These questions are written for this pack to test the ideas above. They are not past examination questions and do not predict what your lecturer will set. Answer first, then compare.

1. What information must a patient understand before giving valid consent?

Answer: The nature of the procedure, its purpose, material risks and benefits, alternatives, and that they may refuse or withdraw.

2. Apply the principle of autonomy to a patient who refuses a blood transfusion.

Answer: A competent adult may refuse treatment even if refusal risks death. The team should explain consequences, explore reasons and document the informed refusal.

3. Why should open questions come before closed questions?

Answer: Open questions allow the patient to reveal concerns the clinician did not anticipate, improving diagnostic accuracy and rapport.

4. List three elements of a complete drug history.

Answer: Current medications including doses, adherence, allergies and adverse reactions, and over-the-counter or herbal use.

5. Give one way a student can breach confidentiality unintentionally.

Answer: Discussing a patient in a public space or sharing clinical images on a personal phone or social-media group.

Your 7-step action plan

Tick each step only when you have done it. A tick you cannot defend is worse than an empty box.

- [ ] Practise a full history on two friends and time yourself under ten minutes.
- [ ] Write definitions of the four principles of biomedical ethics with an example each.
- [ ] Read the current MDCN code of conduct and list five key duties.
- [ ] Draft your own consultation opening and closing phrases.
- [ ] Explain informed consent in a three-sentence answer from memory.
- [ ] Set up a spaced-repetition routine for anatomy and physiology.
- [ ] Shadow a clinical posting and write a one-page reflection.

Keep going

One pack will not carry a semester. Pair this with your lecture notes, tutorials, and the past questions your department actually releases. If a topic still feels unclear, take a specific question to your lecturer or course rep - "I do not understand the difference between X and Y" gets a better answer than "I do not understand this course".

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YOUR WORKSPACE

Notes, questions and things to ask

Use this page in class or during revision. Write questions you could not answer instead of copying slides.

Three ideas I can now explain without notes:

A question I still cannot answer:

For my lecturer or course rep:

Next revision action and date: